Lutetium-177 oxodotreotide (Lutathera / peptide receptor radionuclide therapy (PRRT))

Specialist receptor-targeted radiation for selected advanced neuroendocrine tumours (NETs).

Progressive, unresectable or metastatic, well-differentiated G1/G2 somatostatin-receptor-positive gastroenteropancreatic neuroendocrine tumour (NET) under NCCP 00642a. · NCCP 00642 v2 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Specialist G1/G2 somatostatin receptor (SSTR)-positive gastroenteropancreatic neuroendocrine tumour (GEP-NET) PRRT. Use the service's written appointment, somatostatin analogue (SSA), radiation-safety and emergency instructions.

My overall treatment

Four planned administrations, usually 8 weeks apart; each needs specialist clearance.

1	2	3	4
Treatment 1	Treatment 2	Treatment 3	Treatment 4
Week 0	**Week 8**	**Week 16**	**Week 24**

Illustrative spacing. Each administration needs specialist clearance; dates can change.

Treatment day

Anti-sickness medicine, 4-hour kidney-protective amino acids and a 30–40-minute PRRT infusion. Overnight stay may be needed.

A typical 56-day cycle

Day 1–2	Day 3–21	Day 22–56
Treatment and early observation	Recovery and individual precautions	Tests and next-treatment decision

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before course	**Safety / heart checks** Required by regimen	Counts, kidneys/liver; echocardiogram (heart ultrasound) (ECHO)/multigated acquisition (heart pumping) scan (MUGA); pregnancy assessment where applicable.
Before each	**Bloods / potassium** Required by regimen	Counts, kidneys/liver 2–4 weeks before AND shortly before; potassium before amino acids.
Between	**Bloods / clinic** Required by regimen	Counts, kidneys/liver every 2 weeks. Special scans after each infusion.
After final dose	**Continued bloods** Required by regimen	Every 4 weeks for ≥3 months, then every 3 months for the following year; service may arrange earlier extra tests.

Between treatments

- Follow the written SSA injection and radiation-safety plans; some contact restrictions last up to 3 weeks.
- Urgent help for severe flushing, faintness, wheeze or diarrhoea, especially first 48 hours; report fever, bleeding or reduced urine.

Arrange home/work precautions and actual emergency contacts before admission. Scans for tumour response follow your specialist plan.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Energy / wellbeing

PRRT

Tiredness may last beyond the treatment day. Severe weakness, confusion, seizure or collapse needs urgent assessment.

2 Appetite / sickness

PRRT / amino acids

Nausea and vomiting can occur, especially around treatment. Tell staff early and follow the anti-sickness and fluid plan.

3 Heart / fluids / potassium

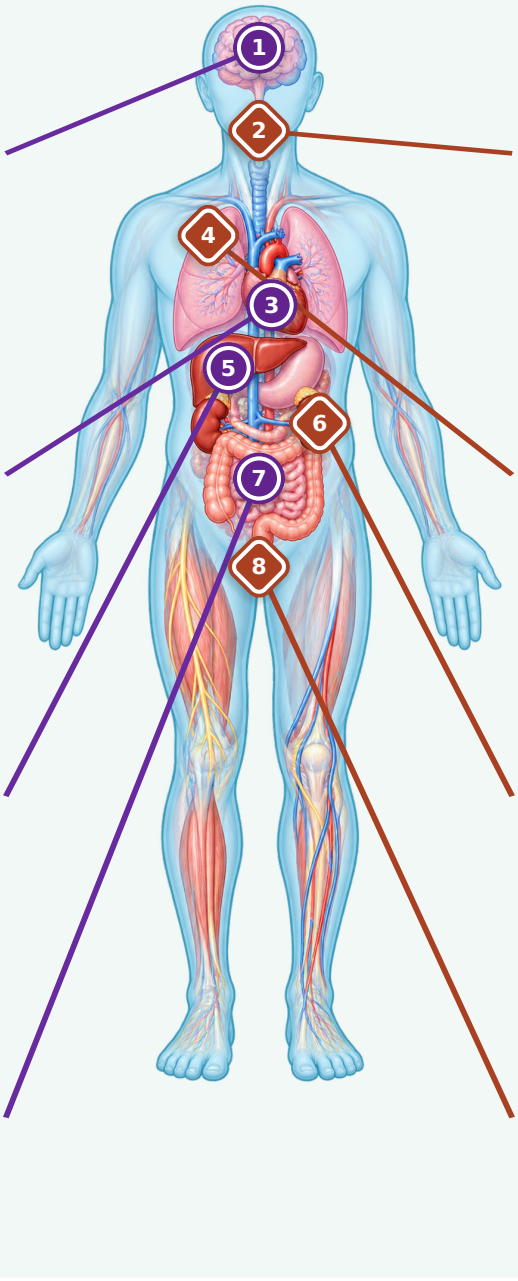
Amino acids / hormone effects

Potassium or fluid changes can affect the heart. Report palpitations, new swelling or breathlessness promptly; severe symptoms are an emergency.

4 Bone marrow / blood

PRRT

Counts may fall during or after the course. Report fever, bruising or bleeding. Rare late marrow disorders require ongoing follow-up.



5 Liver

PRRT

Liver tests are monitored before, between and after treatments. Report yellow skin or eyes, dark urine or persistent severe abdominal pain.

6 Kidneys / urine

PRRT

Amino acids reduce kidney exposure but do not remove risk. Keep renal blood tests and report much less urine or dehydration.

7 Bowel / hormone symptoms

PRRT / NET

Abdominal pain, diarrhoea or sudden hormone release can occur. Severe flushing, faintness, diarrhoea or wheeze needs urgent advice.

8 Fertility / pregnancy

PRRT

Discuss fertility before treatment. Avoid pregnancy using the written contraception plan; follow individual radiation precautions for close contacts.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Seek urgent help for sudden severe flushing with faintness, severe diarrhoea, wheeze or breathing trouble, especially in the first 48 hours; follow the hormone-crisis plan. Fever, bleeding, much less urine, persistent severe abdominal pain, muscle weakness or confusion needs prompt advice. Tell emergency staff when you received PRRT; radiation precautions must not delay urgent care.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

PRRT · Lutathera

Four planned administrations, usually 8 weeks apart. Keep blood tests and written radiation precautions.

Phase: _____ Planned administrations: 4

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My treatment record

Team stamps after each completed treatment.

T1 — Date / initials

T2 — Date / initials

T3 — Date / initials

T4 — Date / initials

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; an administration stamp records that administration. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.